NCCP 00568 · clinician draft

Pembrolizumab + pemetrexed + carboplatin

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Maintenance: pembrolizumab + pemetrexed

Active medicines: pembrolizumab + pemetrexed

Emetogenic risk: low

Medicine	Dose / route	When
Folic acid	350–1000 micrograms orally daily	At least five doses in seven days before first pemetrexed; throughout treatment and 21 days after the final dose
Vitamin B12 (hydroxocobalamin)	1000 micrograms IM	Week before first pemetrexed and every three cycles thereafter Later doses may be on the pemetrexed day.
Pemetrexed rash prophylaxis: dexamethasone equivalent	4 mg orally twice daily	Day before, day of and day after pemetrexed This is the regimen rash schedule; prescriber/pharmacy must reconcile it with antiemetic dexamethasone into one unambiguous prescription, including aprepitant interaction. Do not simply add both courses.
Low-risk antiemetic / rescue	Individual prescription	Before treatment or as required according to current NCCP/local plan Account for the existing rash-prophylaxis steroid. No automatic continuation of induction aprepitant or high-risk multi-day steroid course.

Additional phase-specific support

- Folate, B12 and rash prophylaxis continue because pemetrexed continues.
- Carboplatin ends after induction; review supportive prescriptions at the phase change.
- Continue renal, glucose, thyroid and immune-toxicity monitoring.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

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Sources checked 2026-10-29 · SACTCheck v0.84.27 · Regimen-level drafts; prescriber and oncology-pharmacy verification required